



Assessment Details

Current Initial

Client Demographics

Client Information

Client Name: Ellis, Raysheal M

Client ACES ID: 004240755

Client preferred method of contact: Home Phone

Assessor Name: Donner, Morgan (donnem)

Current Case Manager: Roxby, Amanda (roxbya)

Office: Region 2 HCS/Seattle/King

Assessment Date: 03/09/2026

Reason For Assessment

Client was referred for LTC services by RI International following an administrative transfer from Western State Hospital. Client was civilly committed following an unsuccessful competency restoration related to an index offense alleging Burglary 2, Theft 3, and Resisting Arrest. Client was experiencing homelessness prior to arrest/hospitalization.

Was client the primary source of information? Yes

Other sources of information:

RI International; Western State Hospital

My Goals and Plans

My relationships and interests:

Bunch of family in West Seattle.

How to best support me:

Take a shower, eat something sweet, listen to music - Meegos new pop like Cardi B, exercise.

People can check in with me.

People touching my stuff is a trigger

My perfect day:

Waking up having a smoke and coffee and breakfast, no locked doors, and being able to rest my head somewhere dry.



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What's important to me:

Me taking care of myself, washing dishes, and seeing my family when I need to.

Where I want to live:

Single room, West Seattle preferably or

My spiritual/cultural preferences:

None reported.

In the next year, I really want to:

Staying clean and sober.

Topics of Interest

Category: Employment

Subcategory: Find Employment

Related to: Part time employment

General Strength(s) and Preference(s)

Strengths:

Client is cooperative with caregiver, Can evacuate in emergency, Client has good endurance, Client is motivated

Preferences:

Client expresses preferences, Client prefers to make own decisions

Collateral Contacts

Name: Akujor, Leonard

Relationship: Not related

Role: Facility staff, Hospital staff, Referent, Mental Health Provider, Hospital, Case Manager

Phone: (253)584-2300

Name: RI International

Relationship: Not related

Role: Hospital

Name: Western State Hospital

Relationship: Not related

Role: Hospital



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Name: Ready Meds Pharmacy

Relationship: Not related

Role: Pharmacy

Phone: (877)425-6337, (877)509-6337

Name: Aryyn Integrated Care Clinic

Relationship: Not related

Role: Primary physician

Phone: (253)325-1121, (800)807-9176

Name: Healing Solutions and Integrative Medicine

Relationship: Not related

Role: Mental Health Provider

Phone: (360)888-4476, (800)269-5901

Name: Ndiritu, Grace

Relationship: Not related

Role: Formal Caregiver

Phone: (425)246-4278, (206)429-2182

Name: Chaves, Cecilia

Relationship: Not related

Role: Mental Health Provider

Phone: (360)888-4476

Name: Nwaorgu, Dr. Kelechi

Relationship: Not related

Role: Physician

Phone: (360)888-4476

Name: Nyaribo, Allan

Relationship: Not related

Role: Case Manager

Phone: (360)888-4476



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Name: C & N Angels

Relationship: Not related

Role: Community Choice Guide

Name: Gould, Neveah

Relationship: Sibling

Role: Emergency Contact

Phone: (564)230-8393

Safety

Safety

Caregiver Instruction(s) and Safety Concerns:

Cue to evacuate

AFH Evacuation Level: Independent

Legal Issues

90 day CC

Communication

Speech/Hearing

Modes of expression:



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Speech

Hearing: Hears adequately-normal talk

Vision

Ability to See: Adequate

Limitations:

None of these

Mental/Physical Health

Diagnosis

1. OPIOID DEPENDENCE-REMISS

In controlled environment. Client is currently prescribed suboxone.

2. AMPHETAMIN DEPEND-REMISS

In controlled environment

3. HALLUCINOGEN DEP-REMISS

In controlled environment

4. PSYCHOSIS NOS

Unspecified schizophrenia spectrum disorder not due to a substance of physiological condition

5. IRON DEFIC ANEMIA NOS

6. VITAMIN B DEFICIENCY NOS

7. STOMACH FUNCTION DIS NEC

Functional Dyspepsia

8. DYSURIA

9. IMPACTED CERUMEN



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bilateral

10. ABNORMAL WEIGHT GAIN

Health Indicators:

None of these

Is client comatose? No

Medications

The list of medications was obtained from medical record/client/caregiver on the date of this assessment. Do not use this list as the basis for assistance with or administration of medications.

1. ABILIFY TABLETS
2. CARDURA TABLETS
3. DESCOTE FERROUS SULFATE
4. DOXYCYCLINE
5. MULTIVITAMIN WITH MINERALS
6. OMEPRAZOLE
7. SUBOXONE SUBLINGUAL TABLETS
8. VITAMIN B12

What are the routes?

Sublingual, Oral

At most, how many times per day does the client take medications? 2

Medication Management

Describes the amount of assistance, if any, required to receive prescription medications, over the counter medications, or herbal supplements.



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Self Administration: Assistance required

Frequency of need: Daily, Unmet

Client Limitations:

Does not follow frequency or dosage, Forgets to take medications

Caregiver Instructions:

Hand medication in cup or bowl, Put medications in lockbox, Re-order medications,
Report adverse reactions

Provider
Big Heart Adult Home, LLC - 01

Medical Additional Details

Plans to continue medication, reports finding it helpful.

Health Indicators

Height: 5 feet 5 inches

Weight: 198 pounds

Body Mass Index: 32.94532 Obese

Weight loss: 5% or more in last 30 days; or 10% in last 180 days: No

Weight gain: 5% or more in last 30 days; or 10% in last 180 days: Yes

In general, how would you rate your health?: Excellent

Allergy

Any severe or life threatening allergies: No



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Treatments/Programs/Therapies

Self Directed Care: Individuals who have a functional impairment may direct their Individual Provider to perform a health related task that they would normally be able to perform themselves if they did not have a functional impairment that prevents them from doing so.

Nurse Delegation: In private homes, Adult Family Homes, and in Assisted Living Facilities a Registered Nurse may delegate specific health related tasks to a qualified provider. The tasks are performed as instructed and supervised by the delegating nurse.

Type: Treatments

Name: Supportive Supervision and Oversight

Providers:	
Provider	Frequency
AFH/Assisted Living Facility staff	QD (once daily)

Type: Programs

Name: Alcohol/drug treatment

Providers:	
Provider	Frequency
Mental health	PRN (as needed)

Type: Programs

Name: Mental health therapy/program

Providers:	
Provider	Frequency
Mental health	PRN (as needed)



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Type: Programs

Name: Community integration

Providers:	
Provider	Frequency
Community Choice Guide	PRN (as needed)

CCG for the purchase of transitional items.

General comments

Conditional Release Conditions:

1. Remain at Recovery Innovations E&T or on authorized leave until released on this conditional release.
2. Reside upon release in King County at Big Heart Adult Family Home located at 4230 S. 294th St. Auburn, WA 98001, telephone: (253) 429-2182. The Respondent will follow all rules and/or regulations of the home in which she resides.
3. Allow case manager, Allan Nyaribo, or other assigned case manager with Healing Solutions and Integrative Medicine, in Olympia, WA, telephone: (360) 888-4476, to visit her place of residence unannounced. The Respondent shall not move from this location without notice to and approval of her case manager.
4. Keep all appointments with and follow all treatment recommendations of Dr. Dominic Keya, her primary care provider, or other assigned physician(s) who will monitor her 4medical needs. Dr. Dominic Keya's office is located at 4423 Point Fosdick Dr. Ste. 208-3, Gig Harbor, WA 98335, telephone (253) 325-1121.
5. Keep all appointments and follow all treatment recommendations of Dr. Kelechi Nwaorgu, psychiatrist, or other assigned psychiatrist at Healing Solutions and Integrative Medicine, located at 3435 Martin Way E, Ste. D, Olympia, WA 98506; telephone (360) 888-4476. Dr. Kelechi Nwaorgu will be monitoring the Respondent's 8psychiatric needs.
6. Keep all appointments and follow treatment recommendations of her treatment team including Allan Nyaribo or other designated case manager from Healing Solutions and Integrative Medicine.
7. Take medications exactly as prescribed and cooperate with all laboratory work as requested. Healing Solutions and Integrative Medicine will monitor the Respondent's medication compliance. The Respondent's treatment team will be responsible for deciding when/if she will be voluntarily responsible for maintaining her own medication.
8. Do not substantially deteriorate in her routine level of functioning.
9. Maintain her own health and safety in community.
10. Refrain from any threats or acts of harm to self, others, or property.
11. Abstain from use of alcohol or non-prescribed drugs.
12. Abide by any No Contact Orders/Orders of Protection/Anti-Harassment or Restraining 20Orders issued out of any other court or jurisdiction, including but not limited to any Municipal Court, District Court, Superior or Federal Court.

Client Name: Ellis, Raysheal

Client ACES ID: 004240755

Client ADSA ID: 912171

Assessment Date: 03/09/2026

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13. Work with case manager to resolve any outstanding fees or fines issued out of any other court or jurisdiction, including but not limited to Municipal Court, District Court or Superior Court.

14. Work with case manager and assigned/retained counsel to appear at and resolve any 24 pending criminal, including warrants, or civil legal matters. To the undersigned Petitioner's knowledge, the Respondent's next hearing date, if any, is: NA in NA.

History of Inpatient Psychiatric Hospitalizations:

Smokey Point Behavioral Hospital in 2023

Fairfax in 2024

Western State 9/25/25-Current (client administratively transferred to RI International E&T, still considered a post-Western client).

SUD History:

Records and client both report a history of methamphetamine and opioid (fentanyl) use up to the time of arrest.

Client has also reported use of cocaine, heroin and phencyclidine (PCP) in the past.

Time line, frequency, and first onset unknown.

Client has reported receiving SUD services in the past (both inpatient and outpatient).

Client is open to receiving SUD services upon discharge and reports that her recovery is important to her. She has identified her family as being potential triggers for relapse.

Legal History:

Client's index offense alleges Burglary 2, Theft 3, and Resisting arrest.

Prior convictions:

burglary 2nd degree

criminal trespass 1st degree

malicious mischief 3rd degree

criminal trespass 1st degree.

Sleep

Is provider generally able to get 5 hours of sleep out of an 8 hour period?

Yes

Care planning for sleep:

Naps

Is client satisfied with sleep quality? Yes

Memory



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Is there evidence of short term memory loss? Short term memory is OK

Is there evidence of long term memory loss? Long term memory is OK

Is individual oriented to person? Yes

Cognitive Performance

Comprehension

Expressive: How well is the individual understood by those closest to them, using any means of communication? (By others, the individual is)

Usually Understood

Receptive: How well does the individual understand the information from anyone, by any means of communication? (By individual, others are)

Understands

Decisions

How the client made decisions related to tasks of daily living in the last 7 days:

Difficulty in new situations - The client had an organized daily routine, made decisions related to ADLs in familiar situations, but experienced some difficulty in decision making when faced with new tasks or situations.

Client can become tangential or present with disorganized thinking and requires redirection to stay on topic. This can be further confounded when client is expressing delusional beliefs.

Behavior

Past Behaviors (did not occur in the last 7 days):

Name: Law breaking activities



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Behavior description:

Index offense: Burglary 2, Theft 3, Resisting Arrest. Client is alleged to have been trespassing at a local 7-11 and had supposedly stolen 5 drinks that were in her hands. During the arrest she allegedly pulled her arms towards her chest and pulled away from officers. She also is reported to have needed assistance to exit the patrol car once at jail. Previous legal history: burglary 2nd degree; criminal trespass 1st degree; malicious mischief 3rd degree and criminal trespass 1st degree.

Last occurred: 07/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Client is in a locked 24/7 facility.

Name: Rummages/takes belongings of others

Behavior description:

Oct 2025: Client is noted as having taken the census packet (documents used by staff to record on the status of patients) forcefully from a staff member. Client's index offense alleges she took multiple beverages from a 711 she had previously been trespassed from. Additionally client has prior convictions for burglary, criminal trespass (x2) and malicious mischief.

Last occurred: 10/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Be clear about boundaries around personal space and others' belongings.
Redirect the client away from areas she should not be in.
Gently remind the client of expectations and appropriate behavior.

Name: Assaultive (not during personal care)

Behavior description:

Oct 2025: Client reported attempted to punch a nurse at Western State Hospital. She reports she was upset as "he was supposed to put a paper away that I gave him, he didn't and I got mad, I shouldn't have tried to punch him but my anger gets the best of me" Further notes from WSH ascribe the event to the client's feelings of paranoia. Client's index offense alleges that she resisted arrest (pulled her arms in and pulled away from LEO) while being apprehended for other alleged index offenses.

Last occurred: 10/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Monitor for signs of agitation: raised voice, mood instability, voicing frustration
Ensure the safety of client, other residents, and staff.



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Gently separate the client from the target of her aggression.
Suggest a distracting activity such as listening to music or having a snack.
Give the client space to calm down, but monitor from afar for safety.
Encourage client to take medication and prescribed and report changes to the provider.
If the client is a danger to self or others, call 911.

Name:Hallucinations

Behavior description:

The client has endorsed a history of auditory hallucinations. Most recently she reported hearing voices of family members talking to her about food and eating issues. Client stated "I know not to eat from the garbage can, I don't like wasting food".
Client has also described other unusual perceptions such as "people grow on trees" (Sep 26).

Last occurred: 11/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Monitor for signs of hallucinations such as responding to internal stimuli (talking to unseen others, mumbling/laughing to self, looking off into space intently.
If client appears fearful or distressed, reassure the client that she is safe.
Encourage client to take medication as prescribed.
Report changes to the provider.

Name:Delusions

Behavior description:

Client may present with disorganized or tangential thinking.
Client also experiences paranoid/persecutory delusions that others are out to get her, that she has been assaulted by an individual (in spite of staff being present for the entirety of the interaction in which this is reported to have occurred) or otherwise being aggressive.
Client has also made statements suggesting paranoia towards medical staff, that they are altering her medication with no medical basis.

Last occurred: 01/2026

Is there a current intervention? Addressed with current interventions

Personalized interventions:

No not argue with or engage with delusional statements aside from a brief acknowledgment.
Redirect the conversation to a reality based topic.
Suggest an alternative activity such as listening to music or exercising.
Be aware that paranoid delusions may cause the client to escalate into verbal and physical aggression.
Encourage the client to take medication as prescribed.
Report changes to the provider.



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Name: Easily irritable/agitated requiring intervention

Behavior description:

Client can become easily frustrated, either when she does not understand something, or is given news/information that she does not agree with or is disappointing to her. This leads her to become argumentative and at times, even verbally aggressive/hostile (accusing, cussing, raised voice).

Last occurred: 01/2026

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Utilize active listening and validation to help the client feel that her concerns or frustrations are being heard.

Offer realistic solutions if concerns are realistic, is concerns are a result of delusional thinking, attempt to redirect the conversation towards reality based topics.

Suggest a coping skill such as exercise or listening to music.

Allow the client space to take a break, but monitor for safety.

Encourage client to take medication as prescribed.

Report changes to the provider.

Name: Repetitive physical movement/pacing

Behavior description:

Pacing

Last occurred: 01/2026

Is there a current intervention? No interventions in place

Name: Extreme/rapid mood swings

Behavior description:

Client's emotions can change suddenly, becoming easily upset and frustrated, or else becoming quickly sad and tearful. Client may also exhibit heightened excitement or otherwise incongruent mood when she is discussing delusional topics.

Last occurred: 12/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Utilize active listening and validation to help the client feel that her concerns or frustrations are being heard.

Offer realistic solutions if concerns are realistic, is concerns are a result of delusional thinking, attempt to redirect the conversation towards reality based topics.

Suggest a coping skill such as exercise or listening to music.

Allow the client space to take a break, but monitor for safety.



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Encourage client to take medication as prescribed.
Report changes to the provider.

Name: Unrealistic fears and suspicions

Behavior description:

Client has made allegations that others have assaulted her (despite clear evidence to the contrary) or that they are being aggressive towards her or conspiring against her. Client has also made paranoid statements regarding the administration of her medication (medication being changed when it had not).

Last occurred: 01/2026

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Validate the underlying fear, but do not argue with delusional statements.
Redirect the conversation to a reality based topic.
Suggest an alternative activity such as listening to music or exercising.
Be aware that paranoid delusions may cause the client to escalate into verbal and physical aggression.
Encourage the client to take medication as prescribed.
Report changes to the provider.

Name: Uses offensive language

Behavior description:

Client can become argumentative and verbally aggressive when triggered/upset. Client has been noted to have cussed at peers while at Western State Hospital and required staff redirection to de-escalate.

Last occurred: 11/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Utilize active listening and validation to help the client feel that her concerns or frustrations are being heard.
If concerns are realistic, offer solutions if concerns are a result of delusional thinking, redirect conversation towards reality based topics.
Suggest a coping skill such as exercise or listening to music.
Allow the client space to take a break, but monitor for safety. Verbal aggression may escalate physically.
Encourage client to take medication as prescribed.
Report changes to the provider.

Name: Breaks, throws items

Behavior description:



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The client is noted as having thrown liquid at a staff member and was placed in restraint as a result. Records from Western note that she presented with impulsive and immature behaviors around this time and presented limited understanding of expectations related to discharge. It is unclear if this behavior was impulsive or driven by frustration over not getting what she wants/feels she needs.

Last occurred: 10/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Monitor for signs of agitation: raised voice, mood instability, voicing frustration
Ensure the safety of client, other residents, and staff.
Gently separate the client from the target of her aggression.
Suggest a distracting activity such as listening to music or having a snack.
Give the client space to calm down, but monitor from afar for safety.
Encourage client to take medication as prescribed and report changes to the provider.
If the client is a danger to self or others, call 911.

Name: Intentional self-injury

Behavior description:

Client was on suicide watch while in jail beginning 9/1/25 due to suicidal ideation.
Unclear when suicide watch was discontinued.

Last occurred: 09/2025

Is there a current intervention? Addressed with current interventions

Personalized interventions:

Monitor client for signs that a crisis may be developing such as reports of suicidal ideation, self-harm behaviors, or talking about death or wishing to be dead.
Call the local crisis clinic and follow instructions, be prepared to take the client to the hospital, if client refuses to seek help, call 911.
Encourage client to take medication as prescribed, report all changes to the provider and the client's current CM.

Lacks insight

Coping Skills: Taking a shower, eating something sweet, listening to music - Meegos or new pop like Cardi B, exercise.

Delusions continued: Client is also documented as having experienced delusions of grandeur and greatly overestimating her abilities, specifically around keeping herself safe and stable post discharge. Client has also made statements related to food (see hallucinations) but also that other patients have given her food that she was required to eat.

Community Behavioral Health Support (CBHS) Services/1915i



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Community Behavioral Health Support (CBHS) Services/1915i

Was a referral made for CBHS/1915i eligibility? Yes

Date referral was sent to MCO/HCA: 03/16/2026

Suicide

Reports no history of SI however, client was placed on suicide watch in September 2025 while in jail.

If client is experiencing suicidal warning signs, assist them by contacting a mental health professional, or the National Suicide Prevention Lifeline at 988. If verbalizing suicide plan call 911 or assist client with getting to the nearest emergency room. If the person seems unwilling to accept treatment, call the police, your local hospital emergency department, or 911 if you feel that they are in immediate danger. Continue to monitor process and document additional safety efforts. Contact ECS/SBS/IRT Provider (if applicable). Contact assigned HCS Case Manager.

- i. King County Crisis Line: 1-866-427-4747
- ii. Pierce County Crisis Line: 1-800-576-7764
- iii. Southeast WA Crisis Line: 1-800-576-7764
- iv. North Central Crisis Line: 1-800-852-2923
- v. Warm Line (Peer Support): 1-877-500-9276

Activities of Daily Living (ADLs)

Independence and Improvement

Number of days per week the client usually went out of their residence:

Daily

Overall self-sufficiency has changed significantly as compared to status of 90 days ago: No Change



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Do tasks need to be broken down into individual steps to be accomplished (task segmentation)? No

Does client adjust easily to change in routine? No

Universal Precautions

Universal precautions, also referred to as standard precautions, are practices and the use of equipment during client care, regardless of confirmed infection. Precautions are taken to prevent the spread of infectious agents from person to person and all care providers are encouraged to practice universal precautions, which include the use of personal protective equipment (PPE). PPE is effective at covering clothing and skin when there is risk of exposure to blood, saliva, or other infectious agents. PPE may include disposable gloves, face masks, protective eye wear, face shields, and disposable gowns. One of the most important standard precautions is hand washing. Perform hand washing before and after providing care to a client and when there is risk of you touching a surface which may be contaminated by blood or other bodily fluids. Hand washing should take no less than 20 seconds to perform. If soap and water is not available and your hands are not visibly soiled, alcohol-based hand sanitizers are a good alternative. For additional information on standard precautions, please visit your local health department or the Centers for Disease Control and Prevention websites.

Walk in Room, Hallway, and Rest of Immediate Living Environment

How you walked between locations in your room and immediate living environment.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request

Locomotion in Room and Immediate Living Environment

How you moved between locations in your room and immediate living environment. If you are in a wheelchair, locomotion includes how self-sufficient you were once in your wheelchair.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help



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Caregiver Instructions:

Provide assistance at client's request

Locomotion outside of Immediate Living Environment to Include Outdoors

How you moved to and returned from your immediate living environment, outdoors, and more distant areas. If you are living in a contracted assisted living, enhanced services facility, adult residential care, enhanced adult residential care, enhanced adult residential care-specialized dementia care facility or nursing facility (NF), this includes areas set aside for dining, activities, etc. If you are living in your own home or in an adult family home, locomotion outside immediate living environment including outdoors, includes how you moved to and returned from a patio or porch, backyard, to the mailbox, to see the next-door neighbor, or when accessing your community.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request, AFH Evacuation Level - Independent

Bed Mobility

How you moved to and from a lying position, turned side to side, and positioned your body while in bed, in a recliner, or other type of furniture you slept in.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request

Transfer

How you moved between surfaces, e.g., to/from bed, chair, wheelchair, standing position. Transfer does not include how you moved to/from the bath, toilet, or got in/out of a vehicle.



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Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request

Eating

How you ate and drank, regardless of skill. Eating includes any method of receiving nutrition, e.g., by mouth, tube or through a vein. Eating does not include any set up help you received, e.g., bringing food to you or cutting it up in smaller pieces.

Self Performance and Support Provided in the last 7 days:

Independent, Setup help only

Status and Assistance Available:

Unmet

Client Limitations:

Current swallowing problem

Caregiver Instructions:

Provide assistance at client's request

Provider:

Big Heart Adult Home, LLC - 01

Toilet Use

How you eliminated or toileted, used a commode, bedpan, or urinal, transferred on/off toilet, cleansed, changed pads, managed ostomy or catheter, and adjusted clothes. Toilet use does not include emptying a bedpan, commode, ostomy or catheter bag. This type of set up assistance is considered under the definition of support provided.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help



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Caregiver Instructions:

Provide assistance at client's request

Continence Issues

Bladder control (last 14 days): Continent

Bowel control (last 14 days): Continent

Bowel Pattern (last 14 days):

Constipation

Appliances & Programs (last 14 days):

None of these

Individual management (last 14 days): Does not need or use

Dressing

How you put on, fastened, and took off all items of clothing, including donning/removing prosthesis, splints, either braces or orthotics, or both.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request

Personal Hygiene

How you maintain personal hygiene tasks, such as combing hair, brushing teeth, shaving, applying makeup, washing/drying face, hands (including nail care), and perineum, including menses care. Personal hygiene does not include hygiene in baths and showers.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help



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Caregiver Instructions:

Provide assistance at client's request

Bathing

How you took a full-body bath/shower, sponge bath, and transferred in/out of tub/shower.

Self Performance and Support Provided in the last 7 days:

Independent, No setup or physical help

Caregiver Instructions:

Provide assistance at client's request

Foot Care

Foot Care Needs:	
Foot Care	Status
Toenail trimming	Received

Diabetes, poor circulation to the feet, and/or blood thinning medication require trimming of nails and callouses to be self-directed or done by a family member or health care professional.

Has the client received assistance with trimming toenails in the last 90 days? Yes

Will any assistance be provided with toenail trimming in the future? No

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Skin Care

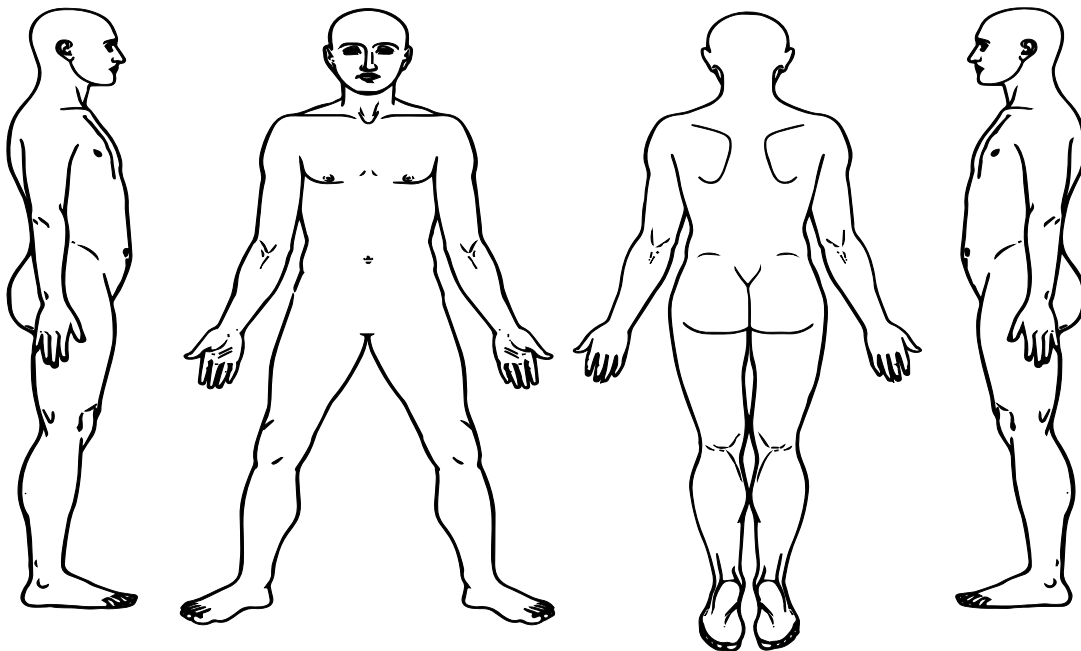
Pressure injuries:

Skin intact over all pressure points

Number of current pressure injuries: 0

Client had skin injury that was resolved or cured in the last year: No

Skin/foot Problem(s)



Instrumental Activities of Daily Living (IADLs)

Client Name: Ellis, Raysheal

Client ACES ID: 004240755

Client ADSA ID: 912171

Assessment Date: 03/09/2026

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Nutritional/Oral

Nutritional Problems:

None of these

Oral hygiene and dental problems:

None of these

Nutritional Approaches:

Diet	Adhere To
None of these	

Date of last dental visit: 02/2026

Community Integration

Does client want to participate in community activities? Yes

Does client need assistance to plan, get to/from, or participate in community activities? Yes

Who will help the client participate in community activities? AFH provider

Will a paid caregiver provide transportation in their own vehicle? Yes

Describe activities the caregiver will support:

Take the client to AA/NA meetings.

Take the client to the library or out shopping in the community.



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Tobacco

Cigarettes pack a day, plans to continue.

Provider Information

Provider Information

Scheduling Preference(s)

The following schedule(s), if identified, are based on the consumer's preference at the time of the assessment.

Informal Providers:

Provider: Healing Solutions and Integrative Medicine **Phone:** (360) 888-4476

Assigned Tasks:
Alcohol/drug treatment, Mental health therapy/program

Formal Providers:

Provider: C N Angels, LLC - 01 **Phone:** (253) 592-2940

Assigned Tasks:
Community integration



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Provider: Big Heart Adult Home, LLC - 01

Phone: (425) 246-4278

Assigned Tasks:

Eating, Med. Mgmt., Supportive Supervision and Oversight

Definitions

ADL Self-Performance Code Definitions

Based on the last 7 Days (after set up has occurred)...

Independent:

No help or oversight OR help/oversight only 1 or 2 times.

Supervision:

Oversight (monitoring, standby) encouragement or cueing provided 3 or more times OR supervision 3 or more times PLUS physical assistance provided only 1-2 times.

Limited Assistance

Client highly involved in activity; received physical help in guided maneuvering of limbs or other non-weight bearing assist 3 or more times OR limited assistance 3 or more times plus weight bearing or full caregiver performance 1 or 2 times.

Extensive Assistance:

While client performed part of activity, help of the following type(s) provided 3 or more times: Weight bearing or full Caregiver performance during part.

Total:

Full Caregiver performance at all times.

Activity did not occur/No provider:

Activity did not occur in entire 7 days because there was no provider available to assist client with task.

Activity did not occur/Client not able:

Activity did not occur in entire 7 days because client is not capable of performing or participating in task.

Activity did not occur/Client declined:

Activity did not occur in entire 7 days because client declined assistance with task.

IADL Self-Performance Code Definitions

- **Independent** – Client received no help, set up or supervision.



Assessment Details

Current Initial

- **Assistance** – Client received any type of assistance, including setup and/or supervision, or activity was fully performed by others.

- **Activity did not occur** – Activity did not occur in the last 30 days.

Independent:

No help, set-up, or supervision.

Set-up help/arrangements only:

On some occasions the client did their own set-up/arrangement; at other times the client received help from another person.

Limited Assistance:

On some occasions the client did not need any assistance but at other times in the last 30 days the client required some assistance.

Extensive Assistance:

Individual involved but required cueing/supervision or partial assistance at all times.

Total dependence:

Activity occurred but with full performance by others.

Activity did not occur